CHAPTER 1



Introduction and Overview

INTRODUCTION

WHERE I STAND

SUGGESTIONS FOR USING THE BOOK

OVERVIEW OF THE THEORY CHAPTERS

INTRODUCTION TO THE CASE OF STAN

- Intake Interview and Stan's Autobiography
- Overview of Some Key Themes in Stan's Life

Copyright 2011 Cengage Learning. All Rights Reserved. May not be copied, scanned, or duplicated, in whole or in part. Due to electronic rights, some third party content may be suppressed from the eBook and/or eChapter(s).
Editorial review has deemed that any suppressed content does not materially affect the overall learning experience. Cengage Learning reserves the right to remove additional content at any time if subsequent rights restrictions require it.

INTRODUCTION

Counseling students can begin to acquire a counseling style tailored to their own personality by familiarizing themselves with the major approaches to therapeutic practice. This book surveys 11 approaches to counseling and psychotherapy, presenting the key concepts of each approach and discussing features such as the therapeutic process (including goals), the client–therapist relationship, and specific procedures used in the practice of counseling. This information will help you develop a balanced view of the major ideas of each of the theories and acquaint you with the practical techniques commonly employed by counselors who adhere to each approach. I encourage you to keep an open mind and to seriously consider both the unique contributions and the particular limitations of each therapeutic system presented in Part 2.

You do not gain the knowledge and experience needed to synthesize various approaches by merely completing an introductory course in counseling theory. This process will take many years of study, training, and practical counseling experience. Nevertheless, I recommend a personal integration as a framework for the professional education of counselors. The danger in presenting one model to which all students are expected to subscribe is that it can limit their effectiveness in working with a diverse range of future clients.

An undisciplined mixture of approaches, however, can be an excuse for failing to develop a sound rationale for systematically adhering to certain concepts and to the techniques that are extensions of them. It is easy to pick and choose fragments from the various therapies because they support our biases and preconceptions. By studying the models presented in this book, you will have a better sense of how to integrate concepts and techniques from different approaches when defining your own personal synthesis and framework for counseling.

Each therapeutic approach has useful dimensions. It is not a matter of a theory being "right" or "wrong," as every theory offers a unique contribution to understanding human behavior and has unique implications for counseling practice. Accepting the validity of one model does not necessarily imply rejecting other models. There is a clear place for theoretical pluralism, especially in a society that is becoming increasingly diverse.

Although I suggest that you remain open to incorporating diverse approaches into your own personal synthesis—or integrative approach to counseling—let me caution that you can become overwhelmed and confused if you attempt to learn everything at once, especially if this is your introductory course in counseling theories. A case can be made for initially getting an overview of the major theoretical orientations, and then learning a particular approach by becoming steeped in that approach for some time, rather than superficially grasping many theoretical approaches. An integrative perspective is not developed in a random fashion; rather, it is an ongoing process that is well thought out. Successfully integrating concepts and techniques from diverse models requires years of reflective practice and a great deal of reading about the various theories. In Chapter 15

Copyright 2011 Cengage Learning. All Rights Reserved. May not be copied, scanned, or duplicated, in whole or in part. Due to electronic rights, some third party content may be suppressed from the eBook and/or eChapter(s).
Editorial review has deemed that any suppressed content does not materially affect the overall learning experience. Cengage Learning reserves the right to remove additional content at any time if subsequent rights restrictions require it.

I discuss in more depth some ways to begin designing your integrative approach to counseling practice.



See the video program for Chapter 1, *DVD for Theory and Practice of Counseling and Psychotherapy: The Case of Stan and Lecturettes*. I suggest that you view the brief lecturette for each chapter in this book prior to reading the chapter.

WHERE I STAND

My philosophical orientation is strongly influenced by the existential approach. Because this approach does not prescribe a set of techniques and procedures, I draw techniques from the other models of therapy that are presented in this book. I particularly like to use role-playing techniques. When people reenact scenes from their lives, they tend to become more psychologically engaged than when they merely report anecdotes about themselves. I also incorporate many techniques derived from cognitive behavior therapy.

The psychoanalytic emphasis on early psychosexual and psychosocial development is useful. Our past plays a crucial role in shaping our current personality and behavior. I challenge the deterministic notion that humans are the product of their early conditioning and, thus, are victims of their past. But I believe that an exploration of the past is often useful, particularly to the degree that the past continues to influence present-day emotional or behavioral difficulties.

I value the cognitive behavioral focus on how our thinking affects the way we feel and behave. These therapies also emphasize current behavior. Thinking and feeling are important dimensions, but it can be a mistake to overemphasize them and not explore how clients are behaving. What people are doing often gives us a good clue to what they really want. I also like the emphasis on specific goals and on encouraging clients to formulate concrete aims for their own therapy sessions and in life. *Contracts* between clients and therapists can be very useful. I frequently suggest either specific *homework assignments* or ask my clients to devise their own assignments, or together we develop goals and tasks that guide the therapy process.

More approaches have been developing methods that involve collaboration between therapist and client, making the therapeutic venture a shared responsibility. This collaborative relationship, coupled with teaching clients ways to use what they learn in therapy in their everyday lives, empowers clients to take an active stance in their world. It is imperative that clients be active, not only in their counseling sessions but in daily life as well. Homework can be a vehicle for assisting clients in putting into action what they are learning in therapy.

A related assumption of mine is that we can exercise increasing freedom to create our future. The acceptance of personal responsibility does not imply that we can be anything that we want. Social, environmental, cultural, and biological realities oftentimes limit our freedom of choice. Being able to choose must be considered in the sociopolitical contexts that exert pressure or create constraints; oppression is a reality that can restrict our ability to choose our future. We are also influenced by our social environment, and much of our behavior is a product of learning and

Copyright 2011 Cengage Learning. All Rights Reserved. May not be copied, scanned, or duplicated, in whole or in part. Due to electronic rights, some third party content may be suppressed from the eBook and/or eChapter(s).
Editorial review has deemed that any suppressed content does not materially affect the overall learning experience. Cengage Learning reserves the right to remove additional content at any time if subsequent rights restrictions require it.

conditioning. That being said, I believe an increased awareness of these contextual forces enables us to address these realities. It is crucial to learn how to cope with the external and internal forces that limit our decisions and behavior.

Feminist therapy has contributed an awareness of how environmental and social conditions contribute to the problems of women and men and how gender-role socialization leads to a lack of gender equality. Family therapy teaches us that it is not possible to understand the individual apart from the context of the system. Both family therapy and feminist therapy are based on the premise that to understand the individual it is essential to take into consideration the interpersonal dimensions and the sociocultural context rather than focusing primarily on the intrapsychic domain. Thus a comprehensive approach to counseling goes beyond focusing on our internal dynamics and addresses those environmental and systemic realities that influence us.

My philosophy of counseling does not include the assumption that therapy is exclusively for the "sick" and is aimed at "curing" psychological "ailments." Such a focus on the medical model restricts therapeutic practice because it stresses deficits rather than strengths. Instead, I agree with the postmodern approaches (see Chapter 13), which are grounded on the assumption that people have both internal and external resources to draw upon when constructing solutions to their problems. Therapists will view these individuals quite differently if they acknowledge that their clients possess competencies rather than pathologies. I view each individual as having resources and competencies that can be discovered and built upon in therapy.

Psychotherapy is a process of engagement between two people, both of whom are bound to change through the therapeutic venture. At its best, this is a collaborative process that involves both the therapist and the client in co-constructing solutions to concerns. Most of the theories described in this book emphasize the collaborative nature of the practice of psychotherapy.

Therapists are not in business to change clients, to give them quick advice, or to solve their problems for them. Instead, counselors facilitate healing through a process of genuine dialogue with their clients. The kind of person a therapist is remains the most critical factor affecting the client and promoting change. If practitioners possess wide knowledge, both theoretical and practical, yet lack human qualities of compassion, caring, good faith, honesty, presence, realness, and sensitivity, they are more like technicians. In my judgment those who function exclusively as technicians do not make a significant difference in the lives of their clients. It is essential that counselors explore their own values, attitudes, and beliefs in depth and work to increase their own awareness. Throughout the book, I encourage you to find ways to personally relate to each of the therapies. Applying this material to yourself personally takes you beyond a mere academic understanding of theories.

With respect to mastering the techniques of counseling and applying them appropriately and effectively, it is my belief that you are your own very best technique. Your reactions to your clients, including sharing how you are affected in the relationship with them, are useful in moving the therapeutic process along.

Copyright 2011 Cengage Learning. All Rights Reserved. May not be copied, scanned, or duplicated, in whole or in part. Due to electronic rights, some third party content may be suppressed from the eBook and/or eChapter(s).
Editorial review has deemed that any suppressed content does not materially affect the overall learning experience. Cengage Learning reserves the right to remove additional content at any time if subsequent rights restrictions require it.

It is impossible to separate the techniques you use from your personality and the relationship you have with your clients.

Administering techniques to clients without regard for the relationship variables is ineffective. Techniques cannot substitute for the hard work it takes to develop a constructive client–therapist relationship. Although you can learn attitudes and skills and acquire certain knowledge about personality dynamics and the therapeutic process, much of effective therapy is the product of artistry. Counseling entails far more than becoming a skilled technician. It implies that you are able to establish and maintain a good working relationship with your clients, that you can draw on your own experiences and reactions, and that you can identify techniques suited to the needs of your clients.

As a counselor, you need to remain open to your own personal development and to address your personal problems. The most powerful ways for you to teach your clients is by the behavior you model and by the ways you connect with them. I suggest you experience a wide variety of techniques yourself *as a client*. Reading about a technique in a book is one thing; actually experiencing it from the vantage point of a client is quite another. If you have practiced mindfulness exercises, for example, you will have a much better sense for guiding clients in the practice of becoming increasingly mindful in daily life. If you have carried out real-life homework assignments as part of your own self-change program, you can increase your empathy for clients and their potential problems. Your own anxiety over self-disclosing and addressing personal concerns can be a most useful anchoring point as you work with the anxieties of your clients. The courage you display in your therapy will help you appreciate how essential courage is for your clients.

Your personal characteristics are of primary importance in becoming a counselor, but it is not sufficient to be merely a good person with good intentions. To be effective, you also must have supervised experiences in counseling and sound knowledge of counseling theory and techniques. Further, it is essential to be well grounded in the various *theories of personality* and to learn how they are related to *theories of counseling*. Your conception of the person and the individual characteristics of your client affect the interventions you will make. Differences between you and your client may require modification of certain aspects of the theories. Some practitioners make the mistake of relying on one type of intervention (supportive, confrontational, information giving) for most clients with whom they work. In reality, different clients may respond better to one type of intervention than to another. Even during the course of an individual's therapy, different interventions may be needed at different times. Practitioners should acquire a broad base of counseling techniques that are suitable for individual clients rather than forcing clients to fit one approach to counseling.

SUGGESTIONS FOR USING THE BOOK

Here are some specific recommendations on how to get the fullest value from this book. The personal tone of the book invites you to relate what you are reading to your own experiences. As you read Chapter 2, "The Counselor: Person and Professional,"

Copyright 2011 Cengage Learning. All Rights Reserved. May not be copied, scanned, or duplicated, in whole or in part. Due to electronic rights, some third party content may be suppressed from the eBook and/or eChapter(s).
Editorial review has deemed that any suppressed content does not materially affect the overall learning experience. Cengage Learning reserves the right to remove additional content at any time if subsequent rights restrictions require it.

begin the process of reflecting on your needs, motivations, values, and life experiences. Consider how you are likely to bring the person you are becoming into your professional work. You will assimilate much more knowledge about the various therapies if you make a conscious attempt to apply their key concepts and techniques to your own personal life. Chapter 2 helps you think about how to use yourself as your single most important therapeutic instrument, and it addresses a number of significant ethical issues in counseling practice.

Before you study each therapy in depth in Part 2, I suggest that you at least briefly read Chapter 15, which provides a comprehensive review of the key concepts from all 11 theories presented in this textbook. I try to show how an integration of these perspectives can form the basis for creating your own personal synthesis to counseling. In developing an integrative perspective, it is essential to think holistically. To understand human functioning, it is imperative to account for the physical, emotional, mental, social, cultural, political, and spiritual dimensions. If any of these facets of human experience is neglected, a theory is limited in explaining how we think, feel, and act.

To provide you with a consistent framework for comparing and contrasting the various therapies, the 11 theory chapters share a common format. This format includes a few notes on the personal history of the founder or another key figure; a brief historical sketch showing how and why each theory developed at the time it did; a discussion of the approach's key concepts; an overview of the therapeutic process, including the therapist's role and client's work; therapeutic techniques and procedures; applications of the theory from a multicultural perspective; application of the theory to the case of Stan; a summary and evaluation; suggestions of how to continue your learning about each approach; and suggestions for further reading.

Refer to the preface for a complete description of other resources that fit as a package and complement this textbook, including *Student Manual for Theory and Practice of Counseling and Psychotherapy* and *DVD for Integrative Counseling: The Case of Ruth and Lecturettes*. In addition, in *DVD for Theory and Practice of Counseling and Psychotherapy: The Case of Stan and Lecturettes*, I demonstrate my way of counseling Stan from the various theoretical approaches in 13 sessions and present my perspective on the key concepts of each theory in a brief lecture, with emphasis on the practical application of the theory.

OVERVIEW OF THE THEORY CHAPTERS

I have selected 11 therapeutic approaches for this book. Table 1.1 presents an overview of these approaches, which are explored in depth in Chapters 4 through 14. I have grouped these approaches into four general categories.

First are the *psychodynamic approaches*. *Psychoanalytic therapy* is based largely on insight, unconscious motivation, and reconstruction of the personality. The psychoanalytic model appears first because it has had a major influence on all of the other formal systems of psychotherapy. Some of the therapeutic models are basically extensions of psychoanalysis, others are modifications of analytic concepts and procedures, and still others are positions that emerged as a reaction against

Copyright 2011 Cengage Learning. All Rights Reserved. May not be copied, scanned, or duplicated, in whole or in part. Due to electronic rights, some third party content may be suppressed from the eBook and/or eChapter(s).
Editorial review has deemed that any suppressed content does not materially affect the overall learning experience. Cengage Learning reserves the right to remove additional content at any time if subsequent rights restrictions require it.

TABLE 1.1 Overview of Contemporary Counseling Models

Psychodynamic Approaches	
Psychoanalytic therapy	Founder: Sigmund Freud. A theory of personality development, a philosophy of human nature, and a method of psychotherapy that focuses on unconscious factors that motivate behavior. Attention is given to the events of the first 6 years of life as determinants of the later development of personality.
Adlerian therapy	Founder: Alfred Adler. Key Figure: Following Adler, Rudolf Dreikurs is credited with popularizing this approach in the United States. This is a growth model that stresses assuming responsibility, creating one's own destiny, and finding meaning and goals to create a purposeful life. Key concepts are used in most other current therapies.
Experiential and Relationship-Oriented Therapies	
Existential therapy	Key figures: Viktor Frankl, Rollo May, and Irvin Yalom. Reacting against the tendency to view therapy as a system of well-defined techniques, this model stresses building therapy on the basic conditions of human existence, such as choice, the freedom and responsibility to shape one's life, and self-determination. It focuses on the quality of the person-to-person therapeutic relationship.
Person-centered therapy	Founder: Carl Rogers. Key figure: Natalie Rogers. This approach was developed during the 1940s as a nondirective reaction against psychoanalysis. Based on a subjective view of human experiencing, it places faith in and gives responsibility to the client in dealing with problems and concerns.
Gestalt therapy	Founders: Fritz and Laura Perls. Key figures: Miriam and Erving Polster. An experiential therapy stressing awareness and integration; it grew as a reaction against analytic therapy. It integrates the functioning of body and mind.
Cognitive Behavioral Approaches	
Behavior therapy	Key figures: B. F. Skinner, Arnold Lazarus, and Albert Bandura. This approach applies the principles of learning to the resolution of specific behavioral problems. Results are subject to continual experimentation. The methods of this approach are always in the process of refinement.
Cognitive behavior therapy	Key figure: A. T. Beck founded cognitive therapy, which gives a primary role to thinking as it influences behavior; Judith Beck continues to develop CBT. Donald Meichenbaum is a prominent contributor to the development of cognitive behavior therapy.

Copyright 2011 Cengage Learning. All Rights Reserved. May not be copied, scanned, or duplicated, in whole or in part. Due to electronic rights, some third party content may be suppressed from the eBook and/or eChapter(s). Editorial review has deemed that any suppressed content does not materially affect the overall learning experience. Cengage Learning reserves the right to remove additional content at any time if subsequent rights restrictions require it.

Rational emotive behavior therapy	Key figure: Albert Ellis founded rational emotive behavior therapy, a highly didactic, cognitive, action-oriented model of therapy that stresses the role of thinking and belief systems as the root of personal problems.
Reality therapy	Founder: William Glasser. Key figure: Robert Wubbolding. This short-term approach is based on choice theory and focuses on the client assuming responsibility in the present. Through the therapeutic process, the client is able to learn more effective ways of meeting her or his needs.
Systems and Postmodern Approaches	
Feminist therapy	This approach grew out of the efforts of many women, a few of whom are Jean Baker Miller, Carolyn Zerbe Enns, Oliva Espin, and Laura Brown. A central concept is the concern for the psychological oppression of women. Focusing on the constraints imposed by the sociopolitical status to which women have been relegated, this approach explores women's identity development, self-concept, goals and aspirations, and emotional well-being.
Postmodern approaches	A number of key figures are associated with the development of these various approaches to therapy. Steve de Shazer and Insoo Kim Berg are the co-founders of solution-focused brief therapy. Michael White and David Epston are the major figures associated with narrative therapy. Social constructionism, solution-focused brief therapy, and narrative therapy all assume that there is no single truth; rather, it is believed that reality is socially constructed through human interaction. These approaches maintain that the client is an expert in his or her own life.
Family systems therapy	A number of significant figures have been pioneers of the family systems approach, including Alfred Adler, Murray Bowen, Virginia Satir, Carl Whitaker, Salvador Minuchin, Jay Haley, and Cloé Madanes. This systemic approach is based on the assumption that the key to changing the individual is understanding and working with the family.

psychoanalysis. Many theories of counseling and psychotherapy have borrowed and integrated principles and techniques from psychoanalytic approaches.

Adlerian therapy differs from psychoanalytic theory in many respects, but it can broadly be considered an analytic perspective. Adlerians focus on meaning, goals, purposeful behavior, conscious action, belonging, and social interest. Although Adlerian theory accounts for present behavior by studying childhood experiences, it does not focus on unconscious dynamics.

The second category comprises the *experiential and relationship-oriented therapies*: the existential approach, the person-centered approach, and Gestalt therapy. The *existential approach* stresses a concern for what it means to be fully human.

Copyright 2011 Cengage Learning. All Rights Reserved. May not be copied, scanned, or duplicated, in whole or in part. Due to electronic rights, some third party content may be suppressed from the eBook and/or eChapter(s). Editorial review has deemed that any suppressed content does not materially affect the overall learning experience. Cengage Learning reserves the right to remove additional content at any time if subsequent rights restrictions require it.

It suggests certain themes that are part of the human condition, such as freedom and responsibility, anxiety, guilt, awareness of being finite, creating meaning in the world, and shaping one's future by making active choices. This approach is not a unified school of therapy with a clear theory and a systematic set of techniques. Rather, it is a philosophy of counseling that stresses the divergent methods of understanding the subjective world of the person. The *person-centered approach,* which is rooted in a humanistic philosophy, places emphasis on the basic attitudes of the therapist. It maintains that the quality of the client–therapist relationship is the prime determinant of the outcomes of the therapeutic process. Philosophically, this approach assumes that clients have the capacity for self-direction without active intervention and direction on the therapist's part. Another experiential approach is *Gestalt therapy,* which offers a range of experiments to help clients gain awareness of what they are experiencing in the here and now—that is, the present. In contrast to person-centered therapists, Gestalt therapists tend to take an active role, yet they follow the leads provided by their clients. These approaches tend to emphasize emotion as a route to bringing about change, and in a sense, they can be considered emotion-focused therapies.

Third are the *cognitive behavioral approaches*, sometimes known as the action-oriented therapies, because they all emphasize translating insights into behavioral action. These approaches include reality therapy, behavior therapy, rational emotive behavior therapy, and cognitive therapy. *Reality therapy* focuses on clients' current behavior and stresses developing clear plans for new behaviors. Like reality therapy, *behavior therapy* puts a premium on doing and on taking steps to make concrete changes. A current trend in behavior therapy is toward paying increased attention to cognitive factors as an important determinant of behavior. *Rational emotive behavior therapy* and *cognitive therapy* highlight the necessity of learning how to challenge inaccurate beliefs and automatic thoughts that lead to behavioral problems. These cognitive behavioral approaches are used to help people modify their inaccurate and self-defeating assumptions and to develop new patterns of acting.

The fourth general approach encompasses the *systems and postmodern perspectives*. Feminist therapy and family therapy are systems approaches, but they also share postmodern notions. The systems orientation stresses the importance of understanding individuals in the context of the surroundings that influence their development. To bring about individual change, it is essential to pay attention to how the individual's personality has been affected by his or her gender-role socialization, culture, family, and other systems.

The *postmodern approaches* include social constructionism, solution-focused brief therapy, and narrative therapy. These newer approaches challenge the basic assumptions of most of the traditional approaches by assuming that there is no single truth and that reality is socially constructed through human interaction. Both the postmodern and the systemic theories focus on how people produce their own lives in the context of systems, interactions, social conditioning, and discourse.

In my view, practitioners need to pay attention to what their clients are *thinking, feeling,* and *doing,* and a complete therapy system must address all three of

Copyright 2011 Cengage Learning. All Rights Reserved. May not be copied, scanned, or duplicated, in whole or in part. Due to electronic rights, some third party content may be suppressed from the eBook and/or eChapter(s).
Editorial review has deemed that any suppressed content does not materially affect the overall learning experience. Cengage Learning reserves the right to remove additional content at any time if subsequent rights restrictions require it.

these facets. Some of the therapies included here highlight the role that cognitive factors play in counseling. Others place emphasis on the experiential aspects of counseling and the role of feelings. Still others emphasize putting plans into action and learning by doing. Combining all of these dimensions provides the basis for a comprehensive therapy. If any of these dimensions is excluded, the therapy approach is incomplete.

INTRODUCTION TO THE CASE OF STAN

You will learn a great deal by seeing a theory in action, preferably in a live demonstration or as part of experiential activities in which you function in the alternating roles of client and counselor. An online program (available in DVD format as well) demonstrates one or two techniques from each of the theories. As Stan's counselor, I show how I would apply some of the principles of each of the theories you are studying to Stan. Many of my students find this case history of the hypothetical client (Stan) helpful in understanding how various techniques are applied to the same person. Stan's case, which describes his life and struggles, is presented here to give you significant background material to draw from as you study the applications of the theories. Each of the 11 theory chapters in Part 2 includes a discussion of how a therapist with the orientation under discussion is likely to proceed with Stan. We examine the answers to questions such as these:

- What themes in Stan's life merit special attention in therapy?
- What concepts would be useful to you in working with Stan on his problems?
- What are the general goals of Stan's therapy?
- What possible techniques and methods would best meet these goals?
- What are some characteristics of the relationship between Stan and his therapist?
- How might the therapist proceed?
- How might the therapist evaluate the process and treatment outcomes of therapy?

In Chapter 16 (which I recommend you read early) I present how I would work with Stan, suggesting concepts and techniques I would draw on from many of the models (forming an integrative approach).

A single case illustrates both contrasts and parallels among the approaches. It also will help you understand the practical applications of the 11 models and provide a basis for integrating them. A summary of the intake interview with Stan, his autobiography, and some key themes in his life are presented next to provide a context for making sense of the way therapists with various theoretical orientations might work with Stan. Try to find attributes of each approach that you can incorporate into a personalized style of counseling.

Intake Interview and Stan's Autobiography

The setting is a community mental health agency where both individual and group counseling are available. Stan comes to counseling because of his drinking. He was convicted of driving under the influence, and the judge determined that he needed professional help. Stan recognizes that he does have problems, but he is

Copyright 2011 Cengage Learning. All Rights Reserved. May not be copied, scanned, or duplicated, in whole or in part. Due to electronic rights, some third party content may be suppressed from the eBook and/or eChapter(s).
Editorial review has deemed that any suppressed content does not materially affect the overall learning experience. Cengage Learning reserves the right to remove additional content at any time if subsequent rights restrictions require it.

not convinced that he is addicted to alcohol. Stan arrives for an intake interview and provides the counselor with this information:

> At the present time I work in construction. I like building houses, but probably won't stay in construction for the rest of my life. When it comes to my personal life, I've always had difficulty in getting along with people. I could be called a "loner." I like people in my life, but I don't seem to know how to stay close to people. It probably has a lot to do with why I drink. I'm not very good at making friends or getting close to people. Probably the reason I sometimes drink a bit too much is because I'm so scared when it comes to socializing. Even though I hate to admit it, when I drink, things are not quite so overwhelming. When I look at others, they seem to know the right things to say. Next to them I feel dumb. I'm afraid that people don't find me very interesting. I'd like to turn my life around, but I just don't know where to begin. That's why I went back to school. I'm a part-time college student majoring in psychology. I want to better myself. In one of my classes, Psychology of Personal Adjustment, we talked about ourselves and how people change. We also had to write an autobiographical paper.

That is the essence of Stan's introduction. The counselor says that she would like to read his autobiography. Stan hopes it will give her a better understanding of where he has been and where he would like to go. He brings her the autobiography, which reads as follows:

> Where am I currently in my life? At 35 I feel that I've wasted most of my life. I should be finished with college and into a career by now, but instead I'm only a junior. I can't afford to really commit myself to pursuing college full time because I need to work to support myself. Even though construction work is hard, I like the satisfaction I get when I look at what I have done.
>
> I want to get into a profession where I could work with people. Someday, I'm hoping to get a master's degree in counseling or in social work and eventually work as a counselor with kids who are in trouble. I know I was helped by someone who cared about me, and I would like to do the same for someone else.
>
> I have few friends and feel scared around most people. I feel good with kids. But I wonder if I'm smart enough to get through all the classes I'll need to become a counselor. One of my problems is that I frequently get drunk. This happens when I feel alone and when I'm scared of the intensity of my feelings. At first drinking seemed to help, but later on I felt awful. I have abused drugs in the past also.
>
> I feel overwhelmed and intimidated when I'm around attractive women. I feel cold, sweaty, and terribly nervous. I think they may be judging me and see me as not much of a man. I'm afraid I just don't measure up to being a real *man*. When I am sexually intimate with a woman, I am anxious and preoccupied with what she is thinking about me.
>
> I feel anxiety much of the time. I often feel as if I'm dying inside. I think about committing suicide, and I wonder who would care. I can see my family coming to my funeral feeling sorry for me. I feel guilty that I haven't worked up to my potential, that I've been a failure, that I've wasted much of my time, and that I let people down a lot. I get down on myself and wallow in guilt and feel *very depressed*. At times like this I feel hopeless and that I'd be better off dead. For all these reasons, I find it difficult to get close to anyone.
>
> There are a few bright spots. I did put a lot of my shady past behind me, and did get into college. I like this determination in me—I *want* to change. I'm tired of feeling the way I do. I know that nobody is going to change my life for me. It's up to me to get what I want. Even though I feel scared at times, I like that I'm willing to take risks.
>
> What was my past like? A major turning point for me was the confidence my supervisor had in me at the youth camp where I worked the past few summers. He helped me get my

Copyright 2011 Cengage Learning. All Rights Reserved. May not be copied, scanned, or duplicated, in whole or in part. Due to electronic rights, some third party content may be suppressed from the eBook and/or eChapter(s).
Editorial review has deemed that any suppressed content does not materially affect the overall learning experience. Cengage Learning reserves the right to remove additional content at any time if subsequent rights restrictions require it.

job, and he also encouraged me to go to college. He said he saw a lot of potential in me for being able to work well with young people. That was hard for me to believe, but his faith inspired me to begin to believe in myself. Another turning point was my marriage and divorce. This marriage didn't last long. It made me wonder about what kind of man I was! Joyce was a strong and dominant woman who kept repeating how worthless I was and how she did not want to be around me. We had sex only a few times, and most of the time I was not very good at it. That was hard to take. It made me afraid to get close to a woman. My parents should have divorced. They fought most of the time. My mother (Angie) constantly criticized my father (Frank Sr.). I saw him as weak and passive. He would *never* stand up to her. There were four of us kids. My parents compared me unfavorably with my older sister (Judy) and older brother (Frank Jr.). They were "perfect" children, successful honors students. My younger brother (Karl) and I fought a lot. They spoiled him. It was all very hard for me.

In high school I started using drugs. I was thrown into a youth rehabilitation facility for stealing. Later I was expelled from regular school for fighting, and I landed in a continuation high school, where I went to school in the mornings and had afternoons for on-the-job training. I got into auto mechanics, was fairly successful, and even managed to keep myself employed for 3 years as a mechanic.

I can still remember my father asking me: "Why can't you be like your sister and brother? Why can't you do anything right?" And my mother treated me much the way she treated my father. She would say: "Why do you do so many things to hurt me? Why can't you grow up and be a man? Things are so much better around here when you're gone." I recall crying myself to sleep many nights, feeling terribly alone. There was no talk of religion in my house, nor was there any talk of sex. In fact, I find it hard to imagine my folks ever having sex.

Where would I like to be 5 years from now? What kind of person do I want to become? Most of all, I would like to start feeling better about myself. I would like to be able to stop drinking altogether and still feel good. I want to like myself much more than I do now. I hope I can learn to love at least a few other people, most of all, a woman. I want to lose my fear of women. I would like to feel equal with others and not always have to feel apologetic for my existence. I want to let go of my anxiety and guilt. I want to become a good counselor for kids. I'm not certain how I'll change or even what all the changes are I hope for. I do know that I want to be free of my self-destructive tendencies and learn how to trust people more. Perhaps when I begin to like myself more, I'll be able to trust that others will find something about me to like.

Effective therapists, regardless of their theoretical orientation, would pay attention to suicidal thoughts. In his autobiography Stan says, "I think about committing suicide." At times he doubts that he will ever change and wonders if he'd be "better off dead." Before embarking on the therapeutic journey, the therapist would need to make an assessment of Stan's current *ego strength* (or his ability to manage life realistically), which would include a discussion of his suicidal thoughts.

Overview of Some Key Themes in Stan's Life

A number of themes appear to represent core struggles in Stan's life. Here are some of the statements we can assume that he may make at various points in his therapy and themes that will be addressed from the theoretical perspectives in Chapters 4 through 14:

- Although I'd like to have people in my life, I just don't seem to know how to go about making friends or getting close to people.
- I'd like to turn my life around, but I have no sense of direction.

Copyright 2011 Cengage Learning. All Rights Reserved. May not be copied, scanned, or duplicated, in whole or in part. Due to electronic rights, some third party content may be suppressed from the eBook and/or eChapter(s).
Editorial review has deemed that any suppressed content does not materially affect the overall learning experience. Cengage Learning reserves the right to remove additional content at any time if subsequent rights restrictions require it.

- I want to make a difference.
- I am afraid of failure.
- I know that when I feel alone, scared, and overwhelmed, I drink heavily to feel better.
- I am afraid of women.
- Sometimes at night I feel a terrible anxiety and feel as if I'm dying.
- I often feel guilty that I've wasted my life, that I've failed, and that I've let people down. At times like this, I get depressed.
- I like it that I have determination and that I really want to change.
- I've never really felt loved or wanted by my parents.
- I'd like to get rid of my self-destructive tendencies and learn to trust people more.
- I put myself down a lot, but I'd like to feel better about myself.

In Chapters 4 through 14, I write about how I would apply selected concepts and techniques of the particular theory in counseling Stan. In addition, in these chapters you are asked to think about how you would continue counseling Stan from each of these different perspectives. In doing so, refer to the introductory material given here and to Stan's autobiography as well. To make the case of Stan come alive for each theory, I highly recommend that you view and study the video program, *DVD for Theory and Practice of Counseling and Psychotherapy: The Case of Stan and Lecturettes*. In this video program I counsel Stan from each of the various theories and provide brief lectures that highlight each theory.

Copyright 2011 Cengage Learning. All Rights Reserved. May not be copied, scanned, or duplicated, in whole or in part. Due to electronic rights, some third party content may be suppressed from the eBook and/or eChapter(s).
Editorial review has deemed that any suppressed content does not materially affect the overall learning experience. Cengage Learning reserves the right to remove additional content at any time if subsequent rights restrictions require it.